



The

FIRST 24 HOURS

What to do when your dialysis facility issues a discharge notice.

42 CFR §494.180

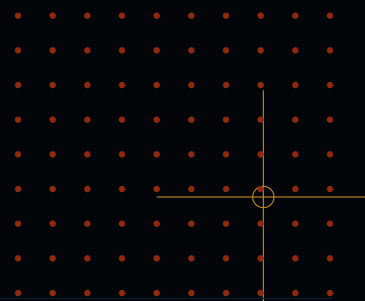
FEDERAL PROTECTION

18 ESRD Networks

US COVERAGE

\$0

PHARMA FUNDING



A PERSONAL NOTE FROM JEFF

Thank you for being here.

I mean that — not as a formality, but as someone who knows exactly what it costs to seek this kind of information in the first place.

If you found Cold Ischemia Foundation, something happened. Something sent you searching for real answers instead of the sanitized language the system hands you. You are in the right place.

This guide was funded by no one but the people who read it. No pharmaceutical partners. No hospital system approval. What you are holding is the guide I needed when I was standing exactly where you are — and it did not exist. Now it does.

If you are in an active crisis right now, turn directly to Section 01. The introduction can wait. Your patient's next treatment window cannot.

ACTIVE CRISIS? GO HERE FIRST:

Section 01 — Hour 0-2 — Your Immediate Rights

Your patient's next scheduled treatment is the first deadline.

Jeff A. Parke

Founder • Cold Ischemia Foundation • Ellenton, Florida

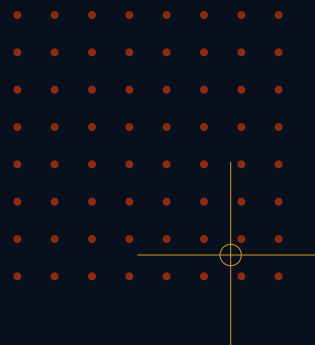
SECTION 01

Your Immediate Legal Rights

The protections that activate before your next treatment.

The moment a discharge notice is issued, federal regulations create a set of rights most care partners are never told about. This section explains what they are, how to invoke them immediately, and what the facility is legally required to do while a dispute is pending.

01



Federal Protections — What Activates Now

01

Right to continued treatment during dispute

Under 42 CFR §494.180(f), a facility cannot discontinue treatment while a formal patient grievance is pending. File the grievance verbally AND in writing before your next scheduled treatment.

02

Right to written notice with specific reasons

The facility must provide written notice stating the specific clinical or behavioral reason for discharge. Vague language — "noncompliance" without specifics — does not satisfy this requirement. Demand the written notice immediately.

03

Right to a 30-day discharge notice period

Except in cases of immediate safety risk, a facility must give 30 days written notice before discharge can take effect. An emergency same-day discharge requires immediate CMS reporting. Know which situation you are in.

42 CFR §494.180(f) — PATIENT GRIEVANCE PROCEDURE

Every ESRD facility is required to maintain and implement a grievance procedure. A patient or representative may file a grievance at any time. The facility must acknowledge within 5 business days and respond in writing within 14 days.

What to Say — What to Demand

WORD-FOR-WORD DISPUTE SCRIPT

"I am formally filing a patient grievance regarding the discharge notice issued to [PATIENT NAME] on [DATE]. I am requesting this grievance be logged immediately, that treatment not be discontinued while the grievance is pending, and that I receive written acknowledgment within 24 hours. I am simultaneously contacting our regional ESRD Network."

WHAT TO DEMAND IN WRITING

- Written acknowledgment of the grievance with date and time stamp
- Specific reasons for the discharge decision in writing
- Name and direct number of the Facility Administrator
- Written confirmation that treatment continues during the grievance
- Name and contact information of your regional ESRD Network

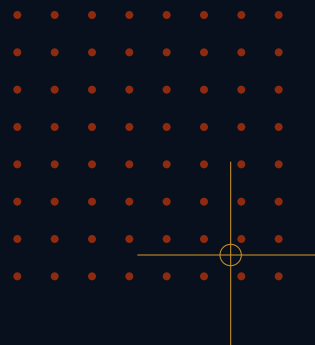
SECTION 02

Contact Your ESRD Network

The most direct path most patients never know exists.

Your regional ESRD Network was created specifically to handle patient grievances with dialysis facilities. Most care partners have never heard of them. This section gives you the direct contact method, the complaint language that gets the fastest response, and the timeline the Network is required to meet.

02



Three Agencies — Contact in This Order

Contact all three. Simultaneously. The same day.

01 · YOUR REGIONAL ESRD NETWORK

Your fastest, most direct path. 18 networks cover all US dialysis patients. They are required to respond to formal grievances and to document every complaint.

PHONE: 1-800-[Find yours: [esrd.network](https://www.esrd.network)]

02 · CMS — CENTERS FOR MEDICARE & MEDICAID SERVICES

A CMS complaint creates a permanent federal record. File online at [QualityNet.cms.gov](https://www.qualitynet.org) or by phone. Reference the discharge notice, treatment dates, and your pending grievance.

PHONE: 1-800-MEDICARE (1-800-633-4227)

03 · YOUR STATE SURVEY AGENCY

The state health department that licenses and inspects dialysis facilities. A state complaint can trigger an unannounced inspection. Find your state agency at [cms.gov/Medicare/Provider-Enrollment](https://www.cms.gov/Medicare/Provider-Enrollment).

PHONE: Varies by state — see [Resources page](#)

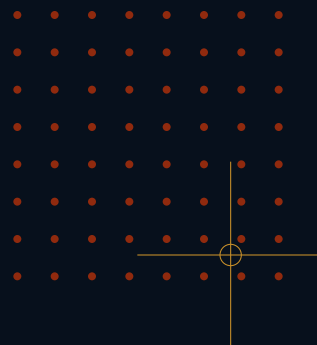
SECTION 03

CMS & State Survey Agency

Two filings. A permanent federal record. State inspection risk.

Filing simultaneously with CMS and your State Survey Agency creates accountability the facility is not prepared for. Most discharge disputes resolve or never reach this point. When they do, facilities that see simultaneous federal and state action understand the situation is no longer manageable through delay.

03



Filing with Federal and State Agencies

Filing simultaneously with CMS and your State Survey Agency creates a paper trail that puts the facility on notice that delay is not a viable strategy. Three pressure points active at the same time — facility grievance, CMS complaint, state agency complaint — changes the power dynamic entirely.

HOW TO FILE WITH CMS

ONLINE

QualityNet.cms.gov — click Submit a Complaint. Free. Instant. Tracked.

PHONE

1-800-MEDICARE (1-800-633-4227) — available 24 hours a day.

WHAT TO INCLUDE

Patient name, facility name and address, date of discharge notice, confirmation that treatment is at immediate risk, and your active grievance reference.

WHAT YOU RECEIVE

A CMS tracking number. Keep it. Reference it in every subsequent filing and every phone call.

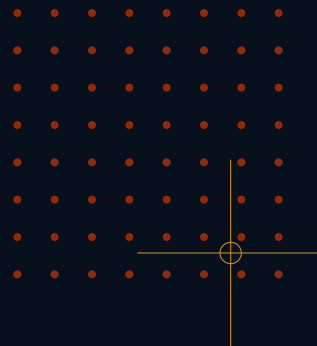
SECTION 04

The Mediation Option

Free. Formal. The tool facilities least want you to request.

Formal ESRD Network mediation places a neutral third party between you and the facility. It is free, available to every dialysis patient, and rarely used because most care partners do not know it exists. The majority of facilities resolve disputes before the scheduled session simply because the request was made.

04



Requesting Mediation — How and Why

THE OPTION MOST PATIENTS NEVER KNOW THEY HAVE

Free formal mediation through your ESRD Network.

Most facilities resolve before the session even starts.

The formal mediation request creates a documented, independent record that neither party can dismiss. When a facility sees that you have filed a grievance, a CMS complaint, a state agency complaint, AND requested formal mediation — all within 24 hours — the calculus shifts. Most choose negotiation.

The key is doing this in writing, not just verbally, within the first 24 hours while everything else is already in motion.

HOW TO REQUEST MEDIATION

- 1 Call your ESRD Network and say: 'I am requesting formal mediation of a patient discharge dispute under the grievance procedure.'
- 2 Follow the call with a written request — email or fax. This creates a timestamped record the facility will see.
- 3 Reference your active grievance and your CMS complaint tracking number. The Network will coordinate with CMS.
- 4 The facility is notified that mediation has been formally requested. This is the moment most facilities begin negotiating directly.

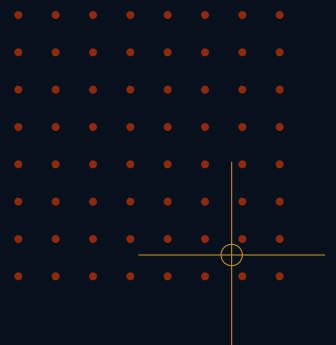
SECTION 05

Staying Functional During the Fight

You cannot win this fight if you cannot function inside it.

This is the section no legal toolkit ever includes. A facility discharge notice in an active care situation triggers a genuine physiological stress response. This section is not wellness advice — it is tactical information required to execute every step in this guide under real crisis conditions.

05



What Helps. What Hurts. What Matters.

A facility discharge notice in an active care situation is a form of institutional trauma. It triggers the same stress response as a physical threat. Your brain will try to catastrophize, freeze, or flee. None of those states allow you to execute the steps in this guide.

What follows is not self-care advice. It is a tactical framework for staying functional enough to win.

WHAT ACTUALLY HELPS IN THE NEXT 24 HOURS

■ **Narrow to the next two hours only.**

The full 24-hour picture is too large to hold under stress. Execute Hour 0-2. Then Hour 2-6. Time-boxing makes the fight manageable.

■ **Say it out loud to someone.**

Not for advice. Just to externalize it. The act of forming language about a crisis reduces its neurological grip. Text someone what is happening.

■ **Eat and drink something before making calls.**

Low blood sugar amplifies the stress response. You will negotiate and communicate measurably better with basic fuel. This is tactical, not indulgent.

■ **Write everything down.**

Names, times, what was said, who said it. Every note you take is both evidence and nervous system regulation. Documentation requires enough executive function to slow the spiral.

■ **You are allowed to be furious. Not yet.**

Anger is appropriate. Save it. Channel it into the complaint letters. The fight is real. The timing of the emotion matters.

Key Contacts & Federal Resources

Federal contacts and resources referenced throughout this guide.

CMS — ONLINE COMPLAINT PORTAL

[QualityNet.cms.gov](https://qualitynet.cms.gov)

Submit a Complaint · Free · Tracked

CMS — PHONE

1-800-MEDICARE (1-800-633-4227)

24 hours · 7 days

ESRD NETWORK LOCATOR

esrd.network · [find-your-network](#)

18 regional networks covering all 50 states

STATE SURVEY AGENCY DIRECTORY

cms.gov/Medicare/Provider-Enrollment-and-Certification/SurveyCertificationGenInfo

Search your state for the licensing agency

MEDICARE RIGHTS CENTER

medicarerights.org

1-800-333-4114 · Free counseling for Medicare patients

COLD ISCHEMIA FOUNDATION — TOOLS

coldischemia.foundation/members-dashboard

AI complaint generators, records requests, insurance appeals



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*Structural advocacy for dialysis patients, transplant
candidates, and the care partners who carry them.*

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Zero sponsored content. Ever.*

— Jeff A. Parke, Founder

*Not medical or legal advice. For educational purposes only.
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